كلية التمريض
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جامعة حلوان
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Psychatric Emergency

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Introduction of psychiatric emergency.

Psychiatric emergencies are conditions requiring psychiatric interventions. The

Demand for psychiatric emergency services has rapidly increased throughout the world. Since the 1960s, especially in urban areas. Psychiatric emergencies may include attempted suicide, substance abuse, depression, psychosis, violence or other rapid Changes in behavior.

Definition of psychiatric emergency

It is an acute disturbance of behaviour, thought or mood of a patient which if Untreated may lead to harm, either to the individual or toothers In the environment and Need immediate intervention.

Causes of psychiatric emergency

- Alcoholism and use of drugs (38%) .
- Affective disorders (29%) .
- Personality disorder (9%) .
- Organic brain syndrome (6%) .
- Anxiety reaction (8%).
- Schizophrenia (2%).
- Undiagnosed patients (8%).

Types of psychiatric emergencies

- Major emergencies
- Sever anxiety
- Suicide or deliberate self-harm
- Agitated and violent patients
- Overdose or side effects of common psychiatric
- Medications
- Overdose and withdrawal from addicting substances
- Minor emergencies
- Grief reactions
- Rape
- Panic attack
- Disaster

Definition of suicide.

Suicide is death caused by injuring oneself with the intent to die

What is the main cause of student suicide?

Biological factors:

1) The neurotransmitter imbalance principally serotonin, dopamine ,
2) Norepinephrine has been linked with emotional responses.
3) Familial or genetic factors.

Psychogenic factors:

1) Alcoholism and substance abuse.
2) Major depression.
3) Generalized anxiety disorder.
4) Low self-esteem.

Sociological factors:

1. .Loneliness.
2. .Loss of loved person or object.
3. .Financial or occupational difficulties

Risk factors

Risk Factors for suicide can vary widely depending on individual circumstances, but some common factors include

1. Mental health disorders such as depression, bipolar disorder, schizophrenia, or substance abuse disorders.
2. Previous suicide attempts.
3. Family history of suicide or mental health disorders.
4. Traumatic life events such as loss of a loved one, abuse, or financial difficulties.
5. Chronic pain or serious illness.
6. Access to lethal means such as firearms or medications.
7. Social isolation or lack of support.
8. Cultural or religious beliefs that stigmatize seeking help for mental health issues.
9. Bullying or harassment.
10. Impulsive or aggressive tendencies.

Levels of suicide:

1) Suicidal Ideation: Direct or indirect thoughts or fantasies of suicide or self-injurious acts
2) Expressed verbally or through writing .
3) Suicide Threats: Direct verbal or written expressions of intent to commit suicide but
4) Without action
5) Suicide Gestures: Self-directed action that results in no injury or minor injury by persons
6) Who neither intended to end their lives nor expected to die as a result .
7) Suicide Attempts: Serious self-directed actions that sometimes result in minor or major
8) Injury by persons who intend to end their lives or seriously harm themselves.
9) Completed Or Successful Suicides: Deaths of persons who ended their lives by their own
10) Means with conscious intent to die

Common Warning Signs

Talking about:

- Wanting to die
- Great guilt or shame
- Being a burden to others

Feeling:

- Empty, hopeless, trapped, or having no reason to live.
- Extremely sad, more anxious, agitated, or full of rage.
- Unbearable emotional or physical pain.
- Changing behavior, such as:
- Making a plan or researching ways to die.

Social withdrawing.

- Taking dangerous risks such as driving extremely fast.
- Displaying extreme mood swings.
- Using drugs or alcohol more often.

Methods of suicide

- Throwing himself in front of
- Train or metro
- Jumping from a building or
- A height
- Overdose of drug or alcohol
- Drowning
- Cutting
- Hanging
- Firearms
- Hanging
- Pesticide

Medical management:

1. **Hospitalization**

A psychiatric unit is usually the most appropriate treatment setting for seriously suicidal people during an acute crisis.the nursing staff helps to protect these patients from hurting themselves while they carry on the tasks of daily living , such as eating ,hygiene ,exercise and communication.

2. **Electro-convulsive therapy (ECT):**

The major therapy used in treatment of suicidal behavior is electroconvulsive therapy (ECT).

Severely suicidal patients who are depressed need to be treated with electro - convulsive therapy (ECT). The procedure has an overall response rate of 75-85%.

3. **pharmacotherapy:**
   - Antidepressant drugs for depression, mood disorder and panic disorder .
   - Antianxiety drugs for panic disorder and personality disorder .
   - Antipsychotic drugs for schizophrenia and personality disorder .
   - Lithium carbonate for mania symptoms in mood disorder

Nursing Management

Assessment

•Assess the client's suicidal potential, and evaluate the level of suicide precautions at least daily.

•Note any previous suicide attempts and methods.

•Observe for risk factors that may increase the chance of suicide attempt.

•History of suicide attempt by oneself or within the family.

• Assess suicidal thoughts or statements.

• Assess sleep habits .

• Assess history of mood disorders.

Nursing diagnosis

• Risk for suicide related to Poor support system, loneliness or disciplinary .

• Problems, Hopelessness, History of multiple suicide attempts , Suicidal .

• Ideation or plan, Social isolation, Substance abuse, Mood disorder , panic .

• And psychosis as manifested by planning and suicidal behavior

Nursing intervention

✓ Render close patient supervision by sustaining observation or awareness of the patient at all times.

✓ Provide a safe environment .

Weapons and pills should be removed by friends, relatives ,or the nurse.

✓ Present opportunities for the patient to express thoughts, and feelings in a nonjudgmental environment.

✓ Create a verbal or written contract stating that the patient will not act on impulse to do self-harm .

✓ Stay with the patient more often.

✓ Disincline the patient in making decisions during severe stress.

✓ Help the patient with problem-solving in a constructive manner.

✓ Arrange for the client to stay with family or friends .

✓ Educate the patient in the appropriate use of medications to facilitate his or her ability to cope.

✓ Contact family members, arrange for individual and/ or family crisis .

✓ Educate the patient cognitive-behavioral self-management

Evaluation

• The patient is free from injury .

• Suicidal thoughts and feelings are reduced.

Definition of Violence:

Is defined by the World Health Organization in the WRVH as "the intentional use of physical force or power, threatened or actual, against oneself, another person, or against a group or community, that either results in or has a high likelihood of resulting in injury, death, psychological harm, maldevelopment or deprivation

Cause of violence?

1. **Individual**

- •History of violent victimization
- •Attention deficits, hyperactivity, or learning disorders
- •History of early aggressive behavior
- •Involvement with drugs, alcohol, or tobacco
- •Low IQ
- •Poor behavioral control
- •Deficits in social cognitive or information-processing abilities
- •High emotional distress.

2. **Family**

- •Authoritarian childrearing attitudes
- •Harsh, lax, or inconsistent disciplinary practices
- •Low parental involvement
- •Low emotional attachment to parents or caregivers.

3. **Peer and Social**

- •Association with delinquent peers
- •Involvement in gangs
- •Social rejection by peers
- •Lack of involvement in conventional activities
- •Poor academic performance
- •Low commitment to school and school failure.

4. **Community**

- •Diminished economic opportunities
- •High concentrations of poor residents
- •High level of transiency
- •High level of family disruption
- •Low levels of community participation
- •Socially disorganized neighborhoods

Risk factors of Violence

1) Age, more young age.
2) Gender, male sex.
3) History of psychiatric illness.
4) Comorbid substance abuse.
5) Positive symptoms of a psychotic disorder have also been shown to be predictors of violent behaviour.

Warning signs of violence

- Mood swings.
- •Serious physical fighting.
- •Severe angry for minor reasons.
- •Increase in use of drugs or alcohol .
- •Increase in risk-taking behavior.
- •Other self-injurious behaviors or threats of suicide.
- •A detailed plan (time, place, and method) to harm or kill others.
- •Use of firearms and other weapons.

types of violence:

Physical Violence

Physical violence occurs when someone uses a part of their body or an object to control a person's actions.

Psychological violence

Occurs when someone uses threats and causes fear in an individual to gain controlled

Verbal Abuse

Verbal abuse occurs when someone uses language, whether spoken or written, to cause harm to an individual.

Sexual Violence

Sexual violence occurs when a person is forced to unwillingly take part in sexual activity.

Spiritual Violence

Spiritual (or religious) violence occurs when someone uses an individual's spiritual beliefs to manipulate, dominate or control that person.

Financial Abuse

Financial abuse occurs when someone controls an individual's financial resources without the person's consent or misuses those resources.

Emotional Violence

Emotional violence occurs when someone says or does something to make a person feel stupid or worthless.

Cultural Violence

Cultural violence occurs when an individual is harmed as a result of practices that are part of her or his culture, religion or tradition.

Neglect

Neglect occurs when someone has the responsibility to provide care or assistance for an individual but does not

Medical management of violence

1. **Hospitalization**:

A psychiatric unit is usually the most appropriate Treatment setting for seriously violent people during an acute crisis. The nursing staff helps to protect these patients from hurting themselves and others.

2. **Psychopharmacological interventions:**

•Antianxiety and sedative-hypnotic medications.

•Antidepressants: Selective serotonin reuptake inhibitors (SSRIs) Are used to reduce agitation associated with post-traumatic stress and anger attacks.

•Mood stabilizers: Lithium and valproate are useful in reducing violence associated with mania, labile mood and impulsivity.

•Antipsychotics: The most effective and common strategy to manage aggression is the administration of high potency typical Antipsychotic Haloperidol in combination with benzodiazepine orazepam.

3. **Restraints**:

Physical restraint

Is a controversial intervention used in psychiatric emergencies and is typically considered a last resort when other interventions have failed and there is a significant risk of harm to the individual or others.

Physical restraint involves the use of physical force to hold a person still or prevent them from moving.

There are several potential risks associated with physical restraint, including physical injury, emotional trauma, and the exacerbation of underlying mental health conditions. Therefore, it should only be used as a temporary measure and for the shortest amount of time Possible.

Chemical restraints-:

Are medications used to restrict the patient's freedom of movement or for emergency control of behavior

Nursing Management

1) **Nursing Diagnosis**: Risk for Violence
2) **Goals:**
   a. Ensure the safety of the patient and others.
   b. Identify triggers and develop coping strategies to prevent violent outbursts.
   c. Provide education on anger management and conflict resolution.
   d. Encourage the patient to seek help from mental health professionals if needed.

3) **Interventions:**
   1. Assess the patient's history of violence, including triggers and warning signs.
   2. Develop a safety plan with the patient, including identifying safe spaces and people to contact in case of emergency.
   3. Teach the patient relaxation techniques, such as deep breathing or mindfulness, to help manage anger and stress.
   4. Encourage the patient to engage in physical activity or hobbies as a healthy outlet for negative emotions.
   5. Collaborate with the healthcare team to monitor the patient's progress and adjust interventions as needed.
   6. Provide information on community resources and support groups for individuals struggling with violence.

4) **Evaluation:**
   a. Monitor the patient's behavior for signs of improvement or escalation of violent tendencies.
   b. Assess the patient's ability to implement coping strategies and utilize support systems effectively.
   c. Evaluate the effectiveness of education and interventions in reducing the risk of violence.
   d. Review the safety plan with the patient regularly and make any necessary adjustments.

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